

Master 01 — Narrative Companion / ICB Submission

Audience: ICB / commissioner readers, senior local-system stakeholders, OTOS advisors.

Purpose: Explain the human, system and service-journey logic behind the proposal without turning the founder story into universal proof.

Status: DRAFT — needs Dean review.

Priority: P2.

Review needed from: Dean; commissioner / ICB reader; evidence reviewer if external.

Source basis: Master 01 source text, Programme Lock, Programme Plan, Founder Story Usage Preface, START update.

A4 REVIEW PAGE 1 — The question the system keeps not asking

When ADHD is left waiting, addiction does not arrive quietly. It brings chaos, crisis and cost with it.

The question is not whether the system is paying. It is whether it is paying early enough to change the outcome.

For a significant and identifiable group of adults, addiction is not the whole picture. The visible crisis may be alcohol or substance use, relapse, A&E, detox, rehab, probation, breakdown, missed appointments or silence after discharge. The driver underneath may be untreated or inadequately treated ADHD.

That distinction matters.

If ADHD is the driver, then recovery work may be asking the person to use the exact capacities unmanaged ADHD impairs: attention, working memory, impulse control, emotional regulation, planning, delayed reward, routine and repeated practice.

So the system keeps treating the crisis, but not always the condition driving it.

OTOS Continuity™ exists to test whether that order can be changed safely.

The intervention tests whether the **OTOS Continuity Management Layer** and user **Continuity Thread** can support a suspected ADHD-driven addiction cohort:

1. identify adults whose addiction, relapse or repeated crisis may be driven by untreated or inadequately treated ADHD;
2. route the ADHD driver toward qualified clinical assessment and treatment where clinically indicated;
3. provide a non-clinical Continuity Management Layer through which authorised partners can manage their agreed part of continuity;
4. follow the user's whole-person journey over time through a Continuity Thread built from agreed non-clinical events;
5. enable authorised partners to support recovery work, addiction support, CBT, structured support / waiting-well routes, service handoffs and behavioural reinforcement to land;
6. evidence what actually happened;
7. decide whether to continue / continue with changes / rebuild / pause / stop.

This is not a claim that ADHD treatment cures addiction. It is not a claim that all addiction is ADHD-driven. It is a bounded, governed test of whether a specific cohort is being found too late.

Treat the driver. Provide the Layer. Follow the Thread. Evidence what happened.

A4 REVIEW PAGE 2 — Where this comes from

Dean Butler, founder of OTOS Digital Ltd, is not adjacent to this problem. He lived it.

For much of his adult life, alcohol looked like part of a successful, social, high-functioning world. It gave him energy, focus, confidence, relief from boredom and a way to feel present. He did not know he was self-medicating. He did not know the driver was ADHD.

Then functional self-medication crossed an invisible line into dependency and crisis.

The system saw him repeatedly:

- addiction services;
- A&E;
- detox;
- residential rehab;
- mental-health contact;
- police, courts and probation;
- years of ADHD waiting-list failure.

He did the work. He engaged. He completed treatment. He went to fellowship. He tried to recover.

But the ADHD driver was not properly connected into the recovery journey. At residential rehab, his stimulant ADHD medication was removed. The ADHD medication review still did not happen. The connection between addiction, ADHD, crisis and recovery was never made.

The turning point came only after the system route had not connected the ADHD driver into the recovery journey and Dean had to seek private clinical correction.

He went private. His diagnosis was corrected. He was prescribed clinically appropriate ADHD treatment. The recovery work that had been floating around him for years finally had somewhere to land.

OTOS was built after that.

That is why the lived experience is not the sales pitch. It is the design specification.

I am not trying to tell my story forever. I am trying to use it once, properly, so the next person does not have to lose seven years before the right driver is treated.

The Founder Arc is design evidence, not universal clinical proof.

A4 REVIEW PAGE 3 — The mechanism

For the suspected ADHD-driven addiction cohort, alcohol or substances may not begin as “misuse”. They may begin as regulation.

They can temporarily provide:

- focus;
- emotional settling;
- confidence;
- social access;
- relief from internal noise;
- stimulation;
- the ability to begin.

Then the regulator becomes the trap.

Dependence forms. Crisis arrives. Addiction becomes the visible problem. But the untreated or inadequately treated ADHD driver is still underneath, and recovery work keeps asking the person to perform executive-function tasks without the executive-function support required to sustain them.

This is why the order matters.

ADHD treatment may open the learning window where clinically indicated. The Continuity Management Layer helps authorised partners keep recovery, waiting-well support and behavioural reinforcement visible and repeated while clinical decisions remain clinically owned.

CBT, recovery education, structured support / waiting-well routes, addiction work, therapy, peer support and behavioural reinforcement may become more usable once the brain can attend, remember, pause, return and repeat.

The sentence must stay caveated:

- ADHD treatment is clinically owned;
- ADHD treatment is not suitable for everyone;
- medication does not do recovery work;
- medication does not cure addiction;
- continuity is not treatment;
- local evidence must be generated before outcome claims are made.

NIA — Neurologically Informed Addiction — may be used in category, evidence and strategic documents as a founder-originated working term. In first-read NHS / commissioner copy, the main phrase should be **suspected ADHD-driven addiction**.

A4 REVIEW PAGE 4 — What OTOS Continuity does

OTOS Continuity™ provides the non-clinical Continuity Management Layer around the intervention test.

It does not diagnose ADHD.

It does not diagnose NIA.

It does not prescribe.

It does not manage medication.

It does not treat addiction.

It does not deliver therapy.

It does not provide crisis care.

It does not make safeguarding decisions.

It does not replace existing services.

It does not write into clinical records.

It does not make automated clinical decisions.

The Continuity Management Layer is the technical, non-clinical infrastructure partners and the user access. The Continuity Thread is the user's whole-person journey over time, built from agreed non-clinical events. Service lines remain each partner's own records, communication channels, work and responsibilities.

Through the Layer, authorised partners can:

- register a tokenised continuity ID;
- record consent;
- log a touchpoint;
- create a handoff receipt;
- trigger or send partner-approved SMS check-ins and prompts;
- show whether the next step landed;
- record SMS delivery / failure / reply / silence;
- surface silence or drift;
- route a signal to human review;

- generate an evidence pack for commissioners and partners.

The Continuity Management Layer is shared continuity infrastructure, not a shared clinical record. The Continuity Thread follows the user journey through agreed non-clinical events.

Partner teams keep their own records and responsibilities. Clinical teams make clinical decisions. The Layer surfaces agreed continuity signals.

A4 REVIEW PAGE 5 — Why this matters to the ICB

The local system already has much of the care:

- addiction support;
- ADHD services;
- crisis teams;
- community recovery;
- therapy;
- structured support / waiting-well routes;
- GP and VCSE routes;
- assessment tools;
- lived-experience knowledge.

What is missing is the Continuity Management Layer around the specific sequence, and a Continuity Thread that follows the user journey across agreed non-clinical events without merging service records.

The first intervention test asks whether a small, governed, non-clinical layer can make that sequence visible enough to answer a serious local question:

Is the system already spending late on a suspected ADHD-driven addiction cohort whose driver could be clinically recognised, routed toward appropriate clinical assessment and treatment where indicated, supported through authorised partner activity in the Continuity Management Layer, followed through a whole-person Continuity Thread, and evidenced earlier?

The first test is not a rollout. It is not a claim of clinical outcome. It is not national proof.

It is a way to produce decision-grade local evidence.

At the end of the first test, the correct decision is not “declare victory”. The correct decision is:

continue / continue with changes / rebuild / pause / stop.

Review notes

Dean decision needed

- ☒ ~~Confirm whether the founder quote stays in Master 01 or moves to a Founder Arc appendix.~~

Dean note: Keep the founder material controlled. The founder story / clinical-letter material stays behind NDA as a protected evidence anchor, but approved themes and short lines can be referenced where they strengthen the case.

- ☒ ~~Confirm whether NIA appears once as a working category or is removed from the ICB first-read draft.~~

Dean note: Use “ADHD-driven addiction” / “suspected ADHD-driven addiction” for first-read NHS and commissioner copy. NIA can remain in controlled evidence / strategic material as founder-originated working terminology, not as a diagnosis.

- ☐ Confirm whether the title remains “Narrative Companion / ICB Submission”.

Evidence / citation check needed

- ☒ ~~Add evidence citations if this version is circulated externally.~~

Dean note: Tick in principle. External circulation needs citations pulled from Master 09 before publication.

- ☒ ~~Check any ADHD / addiction mechanism claims before use outside review.~~

Dean note: Tick in principle. Mechanism claims are directionally right, but must stay guarded: suspected ADHD-driven addiction, where clinically indicated, clinical teams own assessment / treatment, and OTOS provides non-clinical continuity infrastructure.

Clinical / governance check needed

- ☐ Check “ADHD treatment may open the learning window” phrasing.

Dean note: Leave unticked for now. This needs clinical-team input before approval.

☐ Check clinical boundary wording.

Dean note: Leave unticked for now. This needs clinical / governance input before approval.

Copy approval

☐ Approved

☐ Needs edit

☐ Hold
